



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : امينه احمد عبداللطيف خليل

العنوان : ممممممممممممم

الحى / المدينة : المطرية

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 03-10-2026

رقم البطاقة : 27205161302042 رقم التليفون : 01114280239

العمر (تاريخ الميلاد) : 16-05-1972 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: أمينة أحمد عبد اللطيف خليا
National ID: 27205161302042



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

٢٠٢٤/٠٣
١٣٠٢٠٤٢
رؤية مشرق
اتقى
سوليم محمد عبداللطيف خليل
الطائفة سارية هي ٢٠٣٩/٠٣
٨





جمهورية كوت ديفوار
بشارة يحيى الناصح
أبيه
أحمد عبداللطيف خليل
الناصريه
مركز طوبت مهم - الشرقيه
١٩٧٢/٠٣
٢٧٢٠٥١٦١٣٠٢٠٤٢
KM6379823






م. إدارة مجلس المحققين
لجنة مدونة المرأة المصرية

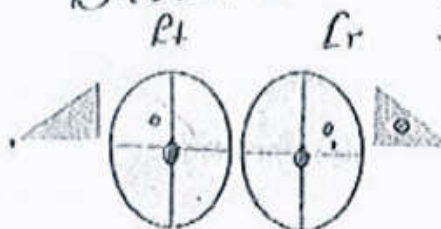
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9.55 - 5 - 10

MDT FORM

Bilateral breast can.

Name	أحمد عبد الله
Age	٤٩
MRN	٥٠٥١/٤١٧٩
ID	٢٧٢٠١٧١٣٠٤٤
Co-morbidity	١١/٤٨٠٠٠٠٠٠



Clinical Data

Dx: Mixed ductal & lobular carcinoma GII		SM: CESM 11/2021 Lt 40p 4.5x6.5x5cm (BIRAD 6) Rt 40p & central largest 8mm lower central mixed 2.5cm firm nipple by US, few of ill defined hypoechoic cysts (BIRAD 5-4)
IHC: ER: 8/8 PR: 8/8 Her2: -ve Ki67: 18%	Rt 7/8 8/8 -ve 15%	Metastasis Workup: CTAP → -ve.
T2 N1 M0 C2 N1 M		
COMMENT: Rt WLE + Rt SLNB → Bifocal invasive lobular carcinoma GII, PNI, no LUJ, LN 0/5 Lt WLE + Lt ALND → Bifocal lobular GII, PNI, LUJ, LN 11/12 lobular & ductal		PT2 N0 PT2 N3

MDT OUTCOME & PLAN

for adjt chenths
4/AC 4 Tante
PORT, formal etc

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
<i>[Signature]</i>		<i>[Signature]</i>	

Name: <u>امينه احمد عبد اللطيف</u>		
MR. : <u>4089</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 53-year-old individual with estrogen-receptor positive, progesterone-receptor positive, HER2-negative breast cancer. Initial management included upfront radiation therapy with wide local excision (WLE) and right sentinel lymph node biopsy (SLNB) showing pathologic stage pT2 N0, followed by left WLE, axillary lymph node dissection (ALND) and pathologic stage pT2 N3 in January 2022. A port was placed and adjuvant chemotherapy was administered. The patient was continued on tamoxifen (Femara) starting in August 2022, with the medication refilled for three months. The patient continued on tamoxifen (Femara) as of August 2022 and was noted to have a pause in therapy for two of five days. Multiple hepatic focal lesions (HFL) were identified on computed tomography (CT) of the abdomen and pelvis with contrast. A computed tomography of the chest, abdomen, and pelvis demonstrated multiple hepatic focal lesions and bone metastases; a bone scan was scheduled for the same day and a multidisciplinary team (MDT) discussion was planned. Bone scan performed in July 2026 revealed progressive bone metastases. The MDT in June 2026 recommended a biopsy of the hepatic focal lesions, which was ordered. A biopsy of the hepatic focal lesions was performed; pathology results are pending and will be available next week. The patient was referred to the pain clinic for symptom management. The patient has metastatic breast cancer with bony pain and will be started on tramadol 50 mg every 12 hours and paracetamol 500 mg every 8 hours for pain control.		
تحريراً في ,		
التاريخ :	20-09-2026	الوقت :
الطبيب المعالج :		علياء احمد أبو النصر شهاب الدين
يعتمد ,		

Patient Name : الأستاذة / أمينة أحمد عبد اللطيف خمار	Request No. : 131108
Patient No. : 27205161302042	Request Date : 07/12/2021
Path No. : 3017/21	Verification Date : 09/12/2021
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Right breast malignant looking mass.

Nature of the specimen:

Tru-cut biopsy.

Gross Examination:

Four grayish white tissue cores up to 1 cm, totally submitted.

- Procedure: Tru-cut biopsy.
- Localization: Right breast.

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade: Nuclear grade II
- Angioinvasion: Negative
- Ductal carcinoma in situ: No.

Diagnosis:

Right breast malignant looking mass; Tru-cut biopsy.

- Invasive duct carcinoma, grade II
- Case is for IPT (ER, PR, Her2, Ki67)

Prof.Dr. Aml Hareedy
Dr.Walaa Ghonaim.

Verified by :
walaa ghoniem

Printing Date : 03/01/2022 09:52:37

Page 1 of 1

Printing By : marwa mohamed youss

المرحلة الأولى من اختبار الكفاءة في الاختصاصات الطبية - 2021
المرحلة الأولى من اختبار الكفاءة في الاختصاصات الطبية - 2021

[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأمينة / أمينة أحمد عبداللطيف خليل	Request No.	: 138046
Patient No.	: 27205161302042	Request Date	: 11/01/2022
Path No.	: 116/2022	Verification Date	: 07/02/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- No lymphovascular emboli
- Ductal carcinoma in situ: No
- Lymph node involvement: All sentinel lymph nodes were free (0/5)
- Resection Margins: All resection margins were free
- Additional pathologic findings: No

Left WLE:

- Histological type: Invasive mixed lobular and ductal carcinoma.
- Grade II (differentiation+ nuclear pleomorphism + mitosis): G (3+2+1)
- Positive perineural invasion and lymphovascular tumour emboli.
- Ductal carcinoma in situ: LCIS (10%)
- Lymph node involvement: Positive metastatic deposits in (11/12) lymph nodes with extensive extracapsular extension.
- Resection Margins: Positive superficial margin. All other margins were free. Extra superficial margin was free.
- Additional pathologic findings: No

Diagnosis:

Rt WLE, Rt sentinel lymph nodes, Lt WLE, Lt extra superficial margin & Lt axillary lymph node dissection:

Right WLE:

Bifocal invasive lobular carcinoma grade II.

Positive perineural invasion

All margins were free, all sentinel lymph nodes were free (0/5)

Stage: pT2(m), N0

Left WLE:

Invasive mixed lobular and ductal carcinoma grade II.

Positive perineural invasion and lymphovascular tumour emboli.

Verified by :

abdelrahman ibrahim

Patient Name	: الأستاذة / أمينة أحمد عبداللطيف خليل	Request No.	: 130022
Patient No.	: 27205161302042	Request Date	: 13/10/2021
Path No.	: 2410/21	Verification Date	: 24/10/2021
Sex	: Female	Financial Category	: ورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referring Doctor		IHC No.	:

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Breast mass

Nature of the specimen: Biopsy

Gross Examination: Five grayish white tissue cores, 1cm each. Totally submitted.

- Procedure: Tru cut
- Localization: Left
- Immunohistochemistry technique: Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), & Ki67 (30-9)

Microscopic examination:

- Histological type: Malignant (Mixed ductal and lobular carcinoma)
- Grade: G II
- Angiolinvasion: No
- Ductal carcinoma in situ: No

Diagnosis:

Left breast, Core biopsy,

- Mixed ductal and lobular carcinoma, grade II.

• Biomarkers:

- o ER: Strong 90% nuclear reaction (Allred score 8/8)
- o PR: Strong 90% nuclear reaction (Allred score 8/8)
- o Her2: No membrane reaction (score 0)
- o Ki67: 18% nuclear reaction

Prof.Dr. Amal Hareedy
Dr. Mona Mohamed

7

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name الأستاذة / أمينة احمد عبداللطيف خليل
Patient No. 27205161302042
Path No. 3017/21
Sex Female
Age 49 Y
Referring Doc.

Request No. 131108
Request Date 07/12/2021
Verification Date 20/12/2021
Financial Category اورام ناز السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Dr. Walaa Ghonaim
Dr. Abdulrahman Ibrahim

Verified by :
abdelrahman ibrahim

Printing Date : 03/01/2022 09:52:48

Page 2 of 2

Printed By : .marwa mohamed yousse

ملف رقمي : 27205161302042 - تاريخ التسجيل : 07/12/2021 - تاريخ التحقق : 20/12/2021
Patient No. 27205161302042 - Path No. 3017/21 - Request No. 131108 - Financial Category : اورام ناز السلام - نفقة الدولة
Region

Patient Name: Ameena Ahmed Abd Al Latif Khallil	Date : 12/11/2022
Exam : C T. ABDOMEN & PELVIS WITH CONTRAST	Age : 53 Year(s)
Medical ID:	Pat. ID: 16104

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of breast carcinoma with no available data.

1. Chest:

- Right breast UOQ irregular speculated mass lesion measuring 3.8 X 3.5 cm.
- Few right pulmonary nodules ranging from 2-3 mm.
- Middle lung lobe reticular opacities and bilateral anterior sub-pleural ground glass veiling.
- Prominent right hilar and retrocaval lymph nodes having sub-centimetric short axis.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Right sided Port-A-cath.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Slightly uterus.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.



Patient Name: Aameena Ahmed Abd Al Latif Khalil	Date : 12/11/2022
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 53 Year(s)
Medical ID.	Pat. ID: 16104

- Element of mesenteric panniculitis.
- 3. The scanned skeleton:
- D8 and L5 sclerotic lesions the largest measuring 9 mm at the former.
- No vertebral structural collapse or extra-osseous soft tissue component.

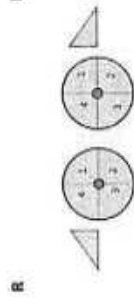
OPINION:

- *Reported case of breast carcinoma.*
- *Few right tiny pulmonary nodules likely benign in nature for follow up.*
- *D8 and L5 sclerotic lesions for bone scan.*
- *No definite hepatic metastatic deposits.*

Dr. Mohammed Mohsen, MSc.
Dr. Amy Farouk, M.D.

جوستاف روتنبرج مستشفى 9 - ٢٧	مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية	05/23/2026
MDT FORM		
Name:	Amina Ahmed Abd Elatif	MRN: 4089
Age:	54 Y	ID: 27205161302042
Menopausal Sta		Phone: * 01142802039
NRS SCORE:		Marital Status:
BMI:		Family History:
Gender		Reason Of MDT:
CO Morbidity:	-	PD
Surgical History: * bil cbs		ECOG Performance Status 0 1 2 3 4

Clinical Data:



BX: PR: + HER2: - KI67:

History:

53 YEARS OLD FEMALE BIL BR CA, ER, PR POSITIVE , HER2 NEGATIVE, UPFRONT RT WLE,
RT SLNB >>> PT2 NO, LT WLE, ALND, PT2 N3 01/2022 >>> RECIEVED
PORT, ADJUVANT CHEMO,
KEPT ON FEMARA SINCE 08/2022
2/5 APUS MULTIPLE HFL
CT CAP MULTIPLE BILOBAR HFLS (Multiple bilobar hypodense
hepatic focal lesions largest measuring 25 x 20 mm segment II) LIKELY
METS
MULTIPLE SMALL VERTEBRAL AND BIL ILIAC BONES FOR BONE SCAN
BONE SCAN > widespread, multi-focal skeletal dissemination (active
metastatic osseous deposits) involving the skull spine, ribs, sternum, and
pelvis.
HFL bx > GATA 3+ MET CA of breast origin, ER 8/8 PR 3/8 HER2 ultralow 0

Clinical Staging:

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

In view of having secondary endocrine resistance, for:

1. 2nd line Faslodex/Palbocidib

2. Zometa q 3 mths

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. نهلة عيسى	د. نهلة عيسى		

about:blank

Patient Name	امينة احمد عبدالطيف	Case No.	347282
Sex/Age	Female 54 Y	Patient No.	25717
Registered Date	10/8/2026 03:16:22 PM	Referred By Dr.	Interventional Radiology
Validation Date	16/9/2026 01:40:40 PM	Branch	Gustave Roussy
Reporting Date	20/9/2026 12:57:26 PM	Pathology No.	2520/26

Pathology report

Clinical Data: Hepatic focal lesion.

Nature of Specimen: Hepatic focal lesion, Tru-cut biopsy, collected on 15/08/2026, at 10:30 am, cold ischemia time: 20 seconds.

Gross Examination:

- Two grey, white tissue cores, ranging in size between 0.2 cm & 1.2 cm long. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & GATA3 (L50-823).

Microscopic Examination:

* Histopathological examination reveals cores of hepatic tissue showing infiltration by tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in clusters with poor tendency to tubular formation amid desmoplastic stroma.

Results:

- * IPT (All controls showed appropriate reactivity):
- GATA3: Strong diffuse positive staining in tumor cells.
- ER: Strong nuclear staining in 98% of tumor cells (score 8/8).
- PR: Weak nuclear staining in 2% of tumor cells (score 3/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).

Diagnosis:

Hepatic focal lesion, Tru-cut biopsy:

Findings are consistent with metastatic carcinoma of breast origin.

IPT:

- GATA3: Positive.
- ER: Positive (score 8/8).
- PR: Positive (score 3/8).
- Her2: Ultra-low (score 0+).



غرفة رقم (4) - شاور الأول - مختبر رقم (987) - كورتيش النيل - مختبر النسيجية - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Courtiche - Alvor El Qudema - Cairo, Egypt

Patient Name	اسميه احمد عبداللطيف	Case No.	347282
Sex/Age	Female 54 Y	Patient No.	25717
Registered Date	10/8/2026 03:16:22 PM	Referred By Dr.	Interventional Radiology
Validation Date	16/9/2026 01:40:40 PM	Branch	Gustave Roussey
Reporting Date	20/9/2026 12:57:26 PM	Pathology No.	2520/26

**** Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

*****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

*Signature**Dr. Walaa Ibrahim Ghonaim, MD.*

مختبر جوستاف رويسى
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mifor El Qadema - Cairo, Egypt
مصر - القاهرة - مبنى رقم (987) - طابق اول - مبنى القنصلية - الطابق 4 - مصر

Name : Amina Ahmed Abdul Latif Khalil

Date : 09-07-2026 08:02

Branch : Dokki

Medical No. : 412353

Ref.Physician Prof : Goustav Roussey Egypt

NUCLEAR MEDICINE DEPARTMENT – Tc-99 MDP WHOLE-BODY

BONE SCINTIGRAPHY

Clinical History: A 54-year-old female patient with a known history of malignant left breast cancer, post-mastectomy, completed chemotherapy and radiotherapy, currently maintained on hormonal therapy. The current study is requested for the evaluation of generalized bone aches.

Comparison: Compared with the previous bone scan report dated January 16, 2023, which revealed no definite evidence of active metastatic osseous deposits.

Technique: Following I.V administration of Tc-99m MDP, anterior and posterior whole-body planar views were acquired approximately 2 hours post-injection.

Scintigraphic Findings

The scanned skeleton reveals widespread, scattered foci of abnormally enhanced radiotracer uptake, which are structurally associated with mixed (sclerotic and lytic) lesions distributed as follows:

Skull: Widespread involvement showing an intense focal area of increased radiotracer uptake in the right skull vault (parieto-frontal region), alongside a small active focus localized at the posterior left occipital bone.

Axial Skeleton:

- **Spine:** Scattered abnormal focal uptake throughout the dorsal and lumbar vertebrae, appearing most prominent within the mid-lumbar vertebral segments.

Name : Amina Ahmed Abdul Latif Khalil

Date : 09-07-2026 08:02

Branch : Dokki

Medical No. : 412353

Ref.Physician Prof : Goustav Roussey Egypt

- **Thoracic Cage:** Multiple foci involving the sternum and scattered bilateral ribs, with a more pronounced distribution noted along the posterior rib aspects.

- **Pelvic Girdle:** Scattered active pelvic osseous lesions.

Impression: Compared to the previous study in January 2023, the current scan demonstrates a significant disease progression characterized by widespread, multi-focal skeletal dissemination (active metastatic osseous deposits) involving the skull, spine, ribs, sternum, and pelvis.

Recommendations: Dedicated radiological correlation of the weight-bearing spine (mid-dorsal region) to assess cortical stability and risk of pathological fracture.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



06/22/2026

MDT FORM

Name:	Amina Ahmed Abd Elatif		MRN:	4089
Age:	54 Y		ID:	27205161302042
Menopausal Sta	POSTMENOPAUSAL		Phone:	01142802039
NRS SCORE:			Marital Status:	
BMI:			Family History:	_VE
CO Morbidity:	_VE		Reason Of MDT:	METS LIVER AND BONE

Clinical Data:

BX:	MIXED G 2		
ER:	8/8	PR:	8/8
		HER2:	0
		KI67:	

Sonomammography:

Mets Workup:

History:

53 YEARS OLD FEMALE , ER , PR POSITIVE , HER2 NEGATIVE , UPFRONT RT WLE , RT SLNB >>> PT2 N 0 , LT WLE , ALND , PT2 N3 01/2022 >> RECEIVED PORT , ADJUVANT CHEMO , KEPT ON FEMARA SINCE 08/2022 2/5 PAUS MULTIPLE HFL

CT CAP MULTIPLE BILOBAR HFLS LIKELY METS

MULTIPLE SMALL VERTEBRAL AND BIL ILIAC BONES FOR BONE SCAN BONE SCAN REQUESTED

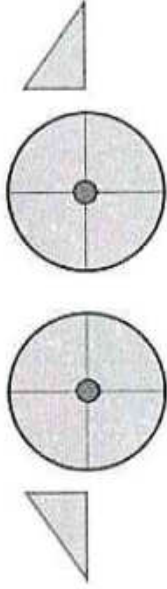
Recommendations:

MDT Outcome & Plan

BIOPSY FROM HFL to confirm hormonal receptors and HER2

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نوراين مصطفى					

د/ نوراين مصطفى
06/22/26



Patient's Name: Amina Ahmed Abdul Latif Khalil
Date: 2026-06-04
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- o No pulmonary nodules, masses or cavitary lesions.
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Enlarged liver showing homogenous parenchymal attenuation. Multiple bilobar hypodense hepatic focal lesions largest measuring 25 x 20 mm at segment II. No dilated biliary radicles.
- o Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o Mesenteric panniculitis.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.
- o Multiple small scattered vertebral and bilateral iliac bones small lytic foci.

Opinion:

- ❖ Multiple bilobar hepatic focal lesions likely metastatic for dynamic MR study.
- ❖ Multiple small vertebral and bilateral iliac bones lytic foci for bone scan.
- ❖ Mesenteric panniculiti

Dr.sherif Wagdy

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National Women's Health Initiative Pathology Lab
معمل باثولوجي الصحة النسائية / المؤسسة الوطنية للصحة النسائية

Patient Name	: الأستاذ / امينة احمد عبد الحفيظ خليل	Request No.	: 130022
Patient No.	: 27205161302042	Request Date	: 13/10/2021
Path No.	: 2410/21	Verification Date	: 24/10/2021
Sex	: Female	Financial Category	: نفقة الزوجة
Age	: 49 Y	Department	: Out Patient
Referring Doctor:		IHC No.	:

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Breast mass

Nature of the specimen: Biopsy

Gross Examination: Five grayish white tissue cores, 1 cm each. Totally submitted.

• Procedure: Tru cut

• Localization: Left

• Immunohistochemistry technique: Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), & Ki67 (30-9)

Microscopic examination:

• Histological type: Malignant (Mixed ductal and lobular carcinoma)

• Grade: G II

• Angioinvasion: No

• Ductal carcinoma in situ: No

Diagnosis:

Left breast, Core biopsy;

• Mixed ductal and lobular carcinoma, grade II.

• Biomarkers:

o ER: Strong 90% nuclear reaction (Allred score 8/8)

o PR: Strong 90% nuclear reaction (Allred score 8/8)

o Her2: No membrane reaction (score 0)

o Ki67: 18% nuclear reaction

Prof.Dr. Amal Hareedy
Dr. Mona Mohamed